4. See the special issue of *Health Systems & Reform* on *financing common goods for health*, September 2019, https://www.who.int/health_financing/topics/financing-common-goods-for-health/en/.
5. Joseph William Bhore, '*Health Survey and Development Committee report*', technical report (Government of India, 1946).
6. Jeffrey Hammer, Yamini Aiyar and Salimah Samji, 'Understanding Government Failure in Public Health Services', *Economic and Political Weekly, Vol.* 42, No. 40 (October 2007), pp. 4049–57, https://www.jstor.org/stable/40276648.
7. The journey from colonial public health to government-funded health insurance is described in Ila Patnaik, Shubho Roy and Ajay Shah, '*The Rise of Government-Funded Health Insurance in India*', technical report 231 (NIPFP, May 2018).
8. For an analysis of the difficulties in health insurance, see Shefali Malhotra et al., '*Fair play in Indian health insurance*', technical report (NIPFP, May 2018), https://macrofinance.nipfp.org.in/releases/MPSS-Fair_play_in_Indian_health_insurance.html.
9. National Health Service, 'GPs per 1000 patients', *NHS*, 2015, https://www.nhs.uk/Scorecard/Pages/IndicatorFacts.aspx?MetricId=100063.

Chapter 36: Financial economic policy

1. Money can be viewed as a public good that facilitates transactions. This function can be equally achieved by US dollars or cryptocurrencies; the creation of the Indian rupee is not essential.

   It is the second dimension of money—reducing business cycle volatility—where the Indian state adds value.

   There is also a third element to money creation: seigniorage income. The central bank earns income on the asset side of its balance sheet, which is paid out to the treasury as a dividend. This income accrues to the Indian government when Indian rupees are used, while this is not the case when the Indian economy transacts through US dollars or cryptocurrencies.
2. B.N. Srikrishna, '*Report of the Financial Sector Legislative Reforms Commission', Vol. 1* (Ministry of Finance, March 2013),